costs as part of future methodology discussions and indicated support for expanding opportunity-based factors within the RHNA framework.

Regional Council Member Gardner asked how county-level income categories used in the RHNA methodology accounted for significant economic differences among communities within large and geographically diverse counties. He noted that income conditions could vary substantially between jurisdictions within the same county and questioned how such local variations were reflected when determining low-, very low-, and other income-level allocations. He emphasized that the treatment of income categories was a key component of the RHNA methodology and sought clarification on how these disparities were addressed.

Ms. Johnson explained that state housing law required income categories used in the RHNA methodology to be based on county median income, regardless of variations among communities within a county. She noted that while county median income was a statutory requirement for establishing income levels, other methodology components developed by SCAG, such as the residual need redistribution process, were discretionary and could be modified as part of the Subcommittee's consideration of future RHNA methodology options.

Regional Council Member Gardner suggested that access to healthcare be considered as a factor in evaluating social equity within the RHNA methodology. He noted that residents in his community had to travel significant distances to access healthcare services and that such services could become inaccessible during severe weather events. Member Gardner emphasized that access to healthcare was an important component of community well-being and recommended that healthcare accessibility be considered when evaluating housing allocations and social equity outcomes.

Ms. Johnson responded that access to healthcare was not a factor included in the social equity indicators used for the sixth-cycle RHNA methodology. She stated that staff would review the current California Tax Credit Allocation Committee (TCAC) opportunity indicators to determine whether healthcare access had been incorporated in more recent updates and noted that it had not been part of the factors considered during the previous RHNA cycle.

Regional Council Member Gardner recommended that access to healthcare be considered as a factor in the development of the seventh-cycle RHNA methodology. He requested that staff evaluate healthcare accessibility as part of its ongoing methodology development efforts, noting that it was an important component of community well-being and social equity. He expressed support for including healthcare access among the factors reviewed as staff refined and developed future RHNA methodology options.

Regional Council Member Lopez expressed appreciation for the discussion and feedback provided on the social equity framework. She stated that she had concerns regarding the inclusion of the